Nephrology Referral Packet Checklist

What Nephrologists Need in a Referral

This document defines what information a nephrology referral packet should contain. Used by the AssembleSpecialtyPacket tool to determine completeness and flag missing items.

Required Items (Critical)

1. eGFR Trend (minimum 2 values, ideally 3+)

- Most recent eGFR with date
- Prior values showing trajectory
- Rate of decline calculated (points per month)
- Duration of decline
- **Why:** Nephrologists need to distinguish acute-on-chronic from progressive CKD. A single value is insufficient.

2. Serum Creatinine Trend

- Corresponding creatinine values for the eGFR measurements
- Important for validating eGFR calculations
- **Why:** eGFR equations can be inaccurate in certain populations; raw creatinine provides a cross-check.

3. Current RAAS Blockade (ACE-I/ARB)

- Drug name and dose
- Duration of use
- Any recent dose changes
- **Why:** ACE-I/ARB management is the primary lever nephrologists use for CKD. They need to know baseline.

4. Active Medication List (full)

- All current medications
- Highlight nephrotoxins: NSAIDs, aminoglycosides, lithium, contrast exposure
- Medications requiring renal dose adjustment
- **Why:** Medication review is a core nephrology intervention.

Important Items (Recommended)

5. Urine Albumin:Creatinine Ratio (ACR)

- Most recent value with date
- If >12 months old: flag as "may need repeating"
- **Why:** Proteinuria determines CKD staging (A1/A2/A3) and influences management. NICE NG203 uses ACR thresholds for referral and SGLT2i initiation.

6. Blood Pressure History

- Last 3-5 readings with dates
- Current antihypertensive regimen
- **Why:** BP target in CKD is <140/90 (or <130/80 with significant proteinuria). Inadequate control accelerates progression.

7. Diabetes Status

- Type, duration, current HbA1c
- Current glucose-lowering regimen
- **Why:** Diabetic nephropathy is the leading cause of CKD. Changes management pathway.

8. Active Conditions List

- All comorbidities
- Highlight conditions affecting kidney management: heart failure, liver disease, autoimmune conditions
- **Why:** Determines suitability for certain treatments and informs risk stratification.

Optional Items (Helpful)

9. Renal Imaging

- Most recent renal ultrasound (if available)
- Findings: size, cortical thickness, obstruction, stones
- **Why:** Rules out obstructive causes and assesses chronicity. Not always required pre-referral.

10. Electrolytes

- Potassium (especially if on ACE-I/ARB or spironolactone)
- Sodium, bicarbonate
- Calcium, phosphate (if eGFR <30)
- **Why:** Electrolyte management becomes critical in advanced CKD.

11. Referral Question

- Specific clinical question for the nephrologist
- Not "please review" but "please assess for CKD progression and advise on medication optimisation"
- **Why:** Focused questions get focused answers.

Completeness Scoring

Score	Interpretation
≥0.8	Excellent — referral is well-supported
0.6-0.8	Adequate — can proceed, some gaps
0.4-0.6	Incomplete — consider addressing gaps first
<0.4	Insufficient — order missing tests before referring

Application to Margaret's Referral:

Available: - eGFR trend: 58 → 52 → 42 (3 values over 5 months) ✓ - Creatinine trend: 95 → 108 → 128 ✓ - Current ACE-I: lisinopril 10mg OD ✓ - Full medication list ✓ - BP: 138/82 (recent) ✓ - Diabetes: T2DM, 10 years ✓ - Conditions: full list ✓

Missing: - Urine ACR: NOT in record — flag as "No urine ACR in last 12 months; consider ordering before referral" - Renal imaging: NOT in record — flag as "No renal imaging on file; not required but may be requested"

Completeness: ~0.75 (adequate for standard referral)